

Level decision

Before you go · Complete after B-1, before any equipment is bought

Decide how far up the tool spectrum this deployment goes — driven by what the hospital can sustain after you leave, not by need.

Hospital	Unit / area	Completed by	Date

1. The stop rule — tick every statement that is TRUE

If ANY box is ticked, drop a level or do not deploy. Dropping a level is not a defeat.

- ☐ No named person is paid to keep the system running after we leave — not 'someone will look after it' — a name, a role, a salary line
- ☐ The medical director has not personally endorsed the project — an administrator's enthusiasm is not the same authority
- ☐ There is no route to fund maintenance for five years — hardware fails, software needs patching, someone must be paid
- ☐ The unit we intend to digitise has no verified working power AND network — verified by us, in the room, with a device
- ☐ We cannot retire the paper process for what we are deploying — if paper continues alongside, nothing has been digitised

2. The tool spectrum

Level	Scope	Tools	Minimum to keep it alive
1	Data collection & surveys	KoBoToolbox, ODK	A trained clerk; no server on a hosted instance
2	Aggregate reporting & case tracking	DHIS2, CommCare	A reporting officer; alignment with the national system
3	Clinical EMR	OpenMRS	A part-time IT person; reliable power where clinicians work
4	Full HIS — clinical, lab, pharmacy, billing	Bahmni, GNU Health	A paid full-time IT person, a funded five-year owner, a support arrangement

3. Decision

Level chosen (1–4, or 'do not deploy')	B-1 total score	Decided by (names)	Date

Justification — why this level and not the one above:

If we deploy below what was hoped: what has to become true for next year's answer to be higher?
